DRUGS · VERTIGO AND MÉNIÈRE'S DISEASE

Vestibular migraine prophylaxis

Vestibular migraine is the commonest cause of recurrent spontaneous vertigo in adults. Treatment combines identifying triggers and lifestyle changes (sleep, meals, hydration, caffeine, analgesic overuse) with migraine prophylaxis when attacks are frequent or disabling. The drugs are those used for migraine headache, chosen by the patient's comorbidities; evidence specific to vestibular migraine is limited.

1 CHAPTER ONE At a glance

The drugs on one page.

1.1 Summary

VESTIBULAR MIGRAINE PROPHYLAXIS

	SUMMARY
Agents	Propranolol; amitriptyline (or nortriptyline); topiramate; pizotifen; candesartan; venlafaxine; flunarizine (not licensed in the UK); CGRP monoclonal antibodies (erenumab, fremanezumab, galcanezumab) for refractory migraine under specialists
Indications	Frequent (for example several a month) or disabling attacks despite lifestyle measures
Adult doses	Propranolol 40 mg twice or three times daily, up to 80 to 160 mg a day; amitriptyline 10 mg at night, increasing to 25 to 50 mg; topiramate 25 mg at night, increasing to 50 mg twice daily; pizotifen 1.5 mg at night; candesartan 8 to 16 mg daily; venlafaxine 75 to 150 mg daily
Trial length	At least 2 to 3 months at an adequate dose before judging

2 CHAPTER TWO Using them in ENT

Choosing a drug.

2.1 Matching drug to patient

- **Propranolol**: good with anxiety or hypertension; avoid in asthma, heart block, and use with care in depression and diabetes.
- **Amitriptyline**: good with insomnia, tension-type headache or neck pain; causes drowsiness, dry mouth and weight gain; cautious in the elderly (anticholinergic, falls, cardiac).
- **Topiramate**: suits overweight patients; causes paraesthesia, cognitive slowing, kidney stones and weight loss; it is **teratogenic**, so women of childbearing potential need a pregnancy prevention programme and highly effective contraception (it also reduces the effect of oral contraceptives).
- **Pizotifen**: causes weight gain and drowsiness.
- **Venlafaxine**: when anxiety or depression coexists.

2.2 Acute treatment

Triptans may help headache; vestibular sedatives for severe vertigo; antiemetics.

3 CHAPTER THREE Safety

What goes wrong.

3.1 Key cautions

Beta blockers mask hypoglycaemia; tricyclics are dangerous in overdose; topiramate and valproate must not be used in pregnancy; sodium valproate is generally avoided in women who could become pregnant.

4 CHAPTER FOUR For the examination

What the examiner wants to hear.

4.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- Diagnostic criteria of vestibular migraine (Bárány Society and International Headache Society).
- The prophylactic drugs and how comorbidities guide choice.
- Topiramate and pregnancy.

4.2 Common pitfalls

COMMON PITFALLS

- Stopping prophylaxis after 2 weeks for lack of effect.
- Prescribing topiramate without a contraception discussion.

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